



The Royal College of Radiologists

Radiotherapy consent form for head and neck cancer (upper sites)

This form should only be used if the patient is over 16 years old and has capacity to give consent. If the patient does not legally have capacity please use an appropriate alternative consent form from your hospital.

Patient details

Patient name:

Date of birth:

Patient unique identifier:

Name of hospital:

Responsible consultant oncologist or consultant therapeutic radiographer:

Special requirements: eg, transport, interpreter, assistance

Details of radiotherapy

Radiotherapy type:	External beam radiotherapy	
Site and side: (Tick as appropriate)	☐ Nasopharynx ☐ Nasal cavity ☐ Sinuses ☐ Salivary glands ☐ Ear ☐ Other _____	Radiotherapy to the neck ☐ Left ☐ Right ☐ Bilateral (both sides)
Aim of treatment: (Tick as appropriate)	☐ Curative – to give you the best chance of being cured ☐ Adjuvant – treatment given after surgery to reduce the risk of cancer coming back ☐ Disease control/palliative – to improve your symptoms and/or help you live longer but not to cure your cancer	
Concurrent systemic anti-cancer therapy: (Tick as appropriate)	☐ Yes with _____ ☐ No (A separate consent form will cover the possible side-effects of this treatment)	

You may have questions before starting, during or after your radiotherapy.

Contact details are provided here for any further queries, concerns or if you would like to discuss your treatment further.

Patient name:

Patient unique identifier:

Possible early or short-term side-effects

Start during radiotherapy or shortly after completing radiotherapy and usually resolve within two to six months of finishing radiotherapy. Frequencies are approximate.

Expected 50%–100% 	☐ Tiredness ☐ Skin soreness, itching, blistering and colour changes in treatment area – white/lighter skin: pink, red, darker than surrounding area; brown skin: maroon or darker than surrounding area; black skin: darker than surrounding area, yellow/purple/grey colour changes ☐ Red or watery eyes ☐ Irritation in the lining of the nose or blocked nose ☐ Mouth ulcers ☐ Pain in the mouth and/or throat which can cause problems with swallowing ☐ Loss or change of taste ☐ Hair loss in treatment area ☐ Anxiety, low mood, feeling fed-up or poor sleep
Common 10%–50% 	☐ Changes in or loss of smell ☐ Temporary hearing loss and/or earache ☐ Dry mouth ☐ Mouth infections including oral thrush ☐ Voice changes ☐ Thickened and tenacious secretions ☐ Nausea – feeling sick ☐ Loss of appetite ☐ Weight loss ☐ Difficulty swallowing which may require temporary placement of a feeding tube at the start of treatment or during treatment to support nutrition and hydration ☐ Risk of hospital admission
Less common Less than 10% 	☐ Chest infection which may be due to food and/or secretions going down the windpipe ☐ Dehydration as a result of reduced oral intake ☐ Vomiting ☐ Lhermitte's sign – temporary changes to the spinal cord presenting as a sudden electric shock like sensation on bending the neck, may occur three to six months after treatment
Rare Less than 1% 	☐ Risk to life
Specific risks to you from your treatment	
I confirm that I have had the above side-effects explained. Patient initials	

Patient name:

Patient unique identifier:

Possible late or long-term side-effects

May happen many months or years after radiotherapy and may be permanent.

Frequencies are approximate.

Expected 50%–100% 	☐ Skin colour change in the treatment area – usually lighter or darker in any skin tone ☐ Lymphoedema – skin, chin and soft-tissue swelling
Common 10%–50% 	☐ Permanent skin texture changes in treatment area – thicker or thinner skin ☐ Telangiectasia in the treatment area – small visible blood vessels which look like spidery marks ☐ Hair loss in the treatment area or patchy re-growth ☐ Permanent dryness of nose ☐ Nasal crusting ☐ Dry mouth ☐ Altered taste or loss of taste – with possibility of some recovery over 18 months ☐ Hypothyroidism – under-active thyroid gland, which may require you to take medication ☐ Cataract – clouding in the lens of the eye, which may require surgery to correct
Less common Less than 10% 	☐ Dry eye ☐ Visual changes and damage to the eye ☐ Nasal regurgitation/reflux ☐ Loss of smell ☐ Hearing loss or hearing changes ☐ Dental problems ☐ Trismus – jaw stiffness ☐ Voice changes ☐ Swallowing problems – with risk of long-term/permanent feeding tube requirement ☐ Increased risk of stroke ☐ Pituitary dysfunction – your pituitary gland not producing enough hormones, this may require you to take medication to replace the hormones ☐ Osteoradionecrosis of the jaw – damage to the jawbone
Rare Less than 1% 	☐ Permanent changes to brainstem, spinal cord and nerves to the face, arm or hand ☐ Radionecrosis of the brain – damage to a small area of the brain which is not repairable ☐ A different cancer in the treatment area ☐ Risk to life
Specific risks to you from your treatment	
I confirm that I have had the above side-effects explained.	
	Patient initials

Statement of health professional

(to be filled in by health professional with appropriate knowledge of proposed procedure)

- I have discussed what the treatment is likely to involve, the intended aims and side-effects of this treatment.
- I have also discussed the benefits and risks of any available alternative treatments including no treatment.
- I have discussed any particular concerns of this patient.

Patient information leaflet provided: ☐ Yes / ☐ No – Details: _____

Copy of consent form accepted by patient: ☐ Yes / ☐ No

Signature: _____

Date: _____

Name: _____

Job title: _____

Statement of patient

- I have had the aims and possible side effects of treatment explained to me and the opportunity to discuss alternative treatment and I agree to the course of treatment described on this form.
- I understand that a guarantee cannot be given that a particular person will perform the radiotherapy. The person will, however, have appropriate expertise.
- I have been told about additional procedures which are necessary prior to treatment or may become necessary during my treatment. This may include permanent skin marks and photographs to help with treatment planning and identification.
- I agree that information collected during my treatment, including images and my health records may be used for education, audit and research. All information will be anonymised. I am aware I can withdraw consent at anytime.

Tick if relevant

- ☐ I confirm that there is no risk that I could be pregnant.
- ☐ I understand that I should not become pregnant during treatment.

Note: if there is any possibility of you being pregnant you must tell your hospital doctor/health professional before your treatment as this can cause significant harm to an unborn fetus. Testosterone and other hormone treatments are not contraception.

- ☐ I understand that if I were to continue to smoke it could have a significant impact on the side-effects I experience and the efficacy of my treatment.

- ☐ I do not have a pacemaker and/or implantable cardioverter defibrillator (ICD).
- or
- ☐ I have a pacemaker and/or implantable cardioverter defibrillator (ICD) and I have had the risks associated with this explained to me.

Signature: _____

Patient name: _____

Date: _____

Statement of:

- ☐ interpreter
- ☐ witness (where appropriate)

- ☐ I have interpreted the information contained in this form to the patient to the best of my ability and in a way in which I believe they can understand.

or

- ☐ I confirm that the patient is unable to sign but has indicated their consent.

Signature: _____

Name: _____

Date: _____

Patient confirmation of consent

(To be signed prior to the start of radiotherapy)

I confirm that I have no further questions and wish to go ahead with treatment.

Patient initials _____

Date: _____